

Q How will my doctor be able to tell how my labor is progressing?

Soon after you arrive at the hospital's labor and delivery ward or the birthing center, if you are planning to use one, you will be given a checkup that will look for several signs that indicate where you are in your labor journey and your general health. Your nurse will take your temperature and blood pressure (and he or she will continue to check these signs every four hours throughout your labor), and you might be asked to give the nurse a urine sample. Between your contractions, your nurse will give you an abdominal examination to make sure your baby's head is engaged, and to assess his position, known as the "presentation" (see p.199) which indicates how your baby is likely to be born. If your nurse thinks that you are in active labor, he or she will do a vaginal examination to confirm this and to see how far your cervix has dilated (see box, opposite). You will be given a vaginal examination about every four hours during your labor to see how your labor is progressing.

Q Will my baby be monitored during labor?

Your baby's heart rate will be monitored throughout labor to assess her well-being. Usually, your baby will be monitored intermittently. The doctor will use a handheld Doppler device, which he or she will place against your abdomen to pick up your baby's heartbeat. If there are any concerns during your labor, you will be advised to have continuous monitoring, using a cardiotocograph (CTG) machine. This machine measures both your baby's heartbeat and the frequency of your contractions. Two devices will be attached to your abdomen using straps, which means you will need to stay close to the monitoring machine. The results are printed out on a graph. If at any point your baby appears to be distressed, your doctor may suggest you have internal monitoring, whereby an electrode attached to a wire is passed through the cervix and placed on your baby's scalp (this doesn't hurt the baby). This is not a routine procedure and your doctor will always discuss with you first the reasons why it is being recommended to you.

Q Can I start pushing as soon as I feel the urge?

It is important that you wait for your doctor to give you the okay to push before you start to bear down. Toward the end of the first stage, when the cervix is around 7-9cm dilated, you move into the transition phase (see box, below). You may feel a strong urge to push now, but if you do so before you are fully dilated, the cervix could swell as the baby's head bears down on it before it's ready to give way to let

the baby pass through, which can make labor much more difficult. If your doctor thinks it's not yet time to push, you may be asked to take little panting breaths followed by a long, slow exhale, which can relieve your urge to bear down. Or, you may be asked to sit and draw your knees up to your chest, another way to ease the need for pushing. Pain relief, such as an epidural, can also slow things down, but your doctor will judge whether this is a good idea now, or if it is better to keep moving forward as you are.

Q I've heard of the stages of labor, but what is "transition"?

The term "transition" describes the end of first stage labor, as your cervix moves toward full dilation, which marks the beginning of second stage labor.

Physiologically, during transition you may feel a strong downward, aching pressure in your lower back, and a heaviness in your perineum as your baby bears down on your cervix. This releases more hormones that cause your cervix to open that final bit—from around 7cm to 9-10cm dilated. However, rather than

being a stage that is mapped by distinct physiological changes, you (and particularly your birth partner and doctor) will know that you have moved into the transition phase of labor because you will undergo some particular behavioral changes, which are listed below.

HOW YOU MIGHT ACT

- » You may start to panic or become suddenly anxious or fearful.
- » You may appear disorientated or confused.
- » You may start yelling or snapping at your birth partner and his or her attempts to calm you.
- » You might demand pain relief when previously you've been confident that you wanted to have a natural birth.
- » You may say that you can't go on.
- » You can move "inward," becoming uncommunicative and perhaps dropping in and out of sleepiness (this is sometimes called the "rest and be thankful" stage).

HOW YOU MIGHT BE FEELING

- » You may feel nauseous or weak.
- » You may feel shivery.
- » It may feel like your contractions are coming more frequently or there is no pause between them.
- » You may feel an urge to push or bear down.
- » There may be bloody discharge.

Your birth partner could learn about the behavioral changes at this stage so that he or she understands your change in attitude!